# PRIOR AUTHORIZATION REQUEST

## Cover Letter

| | |
|---|---|
| Patient Name | John Smith |
| Date of Birth | 1968-03-15 |
| Insurance | United Healthcare |
| Member ID | UHC-887612 |
| Policy Number | UHC-POL-2026-001 |
| CPT Code | 75563 |
| ICD-10 | I42.0 |
| Physician | Dr. Ramesh Kumar |
| Physician NPI | 1234567890 |
| Facility | City Heart Institute |
| Date of Request | March 18, 2026 |

City Heart Institute

Department of Cardiology

123 Medical Center Drive

Houston, Texas 77001

Phone: 555-123-4567

Fax: 555-123-4568

March 18, 2026

United Healthcare Prior Authorization Department

Radiology Prior Authorization Team

[Payer Address - if known, otherwise general department address]

**SUBJECT: PRIOR AUTHORIZATION REQUEST – CARDIAC MRI WITH CONTRAST (CPT 75563)**

**PATIENT: John Smith**

**DATE OF BIRTH: 1968-03-15**

**INSURANCE MEMBER ID: UHC-887612**

**DIAGNOSIS: Dilated Cardiomyopathy (ICD-10: I42.0)**

Dear Prior Authorization Review Team,

This letter serves as a formal request for prior authorization for a Cardiac MRI with contrast (CPT 75563) for our patient, Mr. John Smith.

**Patient Information:**

* **Patient Name:** John Smith

* **Date of Birth:** 1968-03-15

* **Insurance Payer:** United Healthcare

* **Member ID:** UHC-887612

**Requesting Physician Information:**

* **Physician Name:** Dr. Ramesh Kumar

* **NPI:** 1234567890

* **Specialty:** Cardiology

* **Facility:** City Heart Institute

**Procedure Requested:**

* **CPT Code:** 75563

* **Procedure Name:** Cardiac MRI with Contrast

* **Primary Diagnosis:** Dilated Cardiomyopathy (ICD-10: I42.0)

**Medical Necessity Statement:**

Mr. John Smith, a 58-year-old male, has a documented 3-year history of Dilated Cardiomyopathy (ICD-10: I42.0). Despite strict adherence to Guideline-Directed Medical Therapy (GDMT) for over three years, including Carvedilol 25mg twice daily, Lisinopril 10mg daily, Spironolactone 25mg daily, and Furosemide 40mg daily, his symptoms have progressed. He currently presents with New York Heart Association (NYHA) Class III heart failure symptoms, characterized by worsening shortness of breath on exertion, persistent fatigue, significant orthopnea requiring 3-pillow support, and bilateral lower extremity edema. Recent diagnostic findings further underscore the severity of his condition, with a transthoracic echocardiogram from January 15, 2026, revealing a severely reduced Left Ventricular Ejection Fraction (LVEF) of 35%, global hypokinesis, and a moderately dilated left ventricle. Furthermore, a B-type Natriuretic Peptide (BNP) level on March 10, 2026, was significantly elevated at 450 pg/mL, consistent with moderate to severe heart failure. While echocardiography provided a baseline assessment, it is insufficient to fully characterize myocardial tissue, identify specific fibrosis patterns via Late Gadolinium Enhancement (LGE), or definitively assess myocardial viability. A Cardiac MRI with contrast (CPT 75563) is therefore medically necessary to provide high-resolution tissue characterization, differentiate between ischemic and non-ischemic cardiomyopathy, and assess myocardial viability. The detailed information obtained from this advanced imaging study is critical to guide future high-stakes treatment decisions, including potential revascularization or consideration for implantable cardioverter-defibrillator (ICD) placement, and to optimize his long-term management and prognosis.

We have attached supporting clinical documentation, including the physician's order, clinical justification note, echocardiogram report, and recent lab results, for your review. We kindly request your prompt approval for this medically necessary procedure.

Should you require any further information, please do not hesitate to contact our office at 555-123-4567.

Sincerely,

[Prior Authorization Specialist Name]

Prior Authorization Specialist

On behalf of Dr. Ramesh Kumar, MD

NPI: 1234567890

Cardiology

City Heart Institute

# Clinical Summary

## Clinical Summary for Prior Authorization

### 1. Diagnosis:
I42.0: Dilated Cardiomyopathy

### 2. Patient History:
Mr. John Smith, a 58-year-old male, has a documented 3-year history of Dilated Cardiomyopathy, initially diagnosed in January 2023. He presents with worsening New York Heart Association (NYHA) Class III heart failure symptoms, including dyspnea on exertion, persistent fatigue, orthopnea requiring 3-pillow support, and bilateral lower extremity edema. Despite strict adherence to Guideline-Directed Medical Therapy (GDMT) for over three years, his symptoms persist and have shown recent progression.

### 3. Reason for Procedure (CPT 75563 - Cardiac MRI with Contrast):
This procedure is requested to provide definitive myocardial tissue characterization, assess for myocardial fibrosis via Late Gadolinium Enhancement (LGE), differentiate between ischemic and non-ischemic cardiomyopathy, and evaluate myocardial viability. A recent transthoracic echocardiogram (Jan 15, 2026) documented a Left Ventricular Ejection Fraction (LVEF) of 35% but was deemed insufficient to fully characterize myocardial tissue or identify specific fibrosis patterns, which are critical for guiding future high-stakes treatment decisions.

### 4. Supporting Evidence:
* **LVEF:** 35% (Transthoracic Echocardiogram, Jan 15, 2026).

* **BNP:** 450 pg/mL (Lab Results, March 10, 2026), consistent with moderate to severe heart failure.

* **NYHA Class:** III symptoms (Clinical Justification Note, March 16, 2026).

* **Prior Treatments:** Patient has been on maximal tolerated GDMT for 3+ years, including Carvedilol 25mg BID, Lisinopril 10mg daily, Spironolactone 25mg daily, and Furosemide 40mg daily. Sacubitril/Valsartan and Ivabradine were trialed but discontinued due to symptomatic hypotension/dizziness and inadequate heart rate reduction, respectively. Symptoms and LVEF remain unchanged despite optimal medical therapy.

### 5. Clinical Justification:
Cardiac MRI with contrast is a recognized standard of care for comprehensive evaluation of cardiomyopathies when echocardiography is inconclusive or insufficient. This study provides high-resolution data essential for precise myocardial tissue characterization, fibrosis detection, and viability assessment. These findings are crucial for determining prognosis and guiding advanced therapeutic interventions, such as potential revascularization or implantable cardioverter-defibrillator (ICD) placement, in accordance with current

cardiology guidelines.

# Requirements Checklist

| Requirement | Status | Evidence |
|---|---|---|
| Payer Name: United Healthcare Gold Plus | ✔ MET | EHR data specifies 'payer_name: United Healthcare' and 'plan_name: UHC Gold Plus'. |
| CPT Code: 75563 (Cardiac MRI with Contrast) | ✔ MET | EHR data specifies 'cpt_code: 75563'. The Physician Order Letter and Clinical Justification Note also confirm 'Cardiac MRI with Contrast'. |
| Criterion 1: Confirmed diagnosis of Dilated Cardiomyopathy (ICD-10: I42.0), Hypertrophic Cardiomyopathy (I42.1), Restrictive Cardiomyopathy (I42.5), Cardiac Sarcoidosis (D86.85), or Suspected Cardiac Tumor or Mass. | ✔ MET | EHR data specifies 'primary_icd10_code: I42.0' and 'primary_diagnosis: Dilated Cardiomyopathy'. This is also confirmed in the Clinical Justification Note and Physician Order Letter. |
| Criterion 2: Symptoms (Shortness of breath on exertion, Chest pain/pressure, Palpitations/arrhythmia, Unexplained syncope/near-syncope, Reduced exercise tolerance) present and documented for a minimum period of 3 months. | ✔ MET | The Clinical Justification Note dated March 16, 2026, states the patient has a '3-year history of Dilated Cardiomyopathy' and 'current presentation is significant for worsening shortness of breath on exertion, persistent fatigue, and a marked reduction in exercise tolerance... patient reports occasional palpitations'. It also notes 'Despite adherence to optimal medical therapy for over three years, the patient’s symptoms persist'. |
| Criterion 3a: Completed echocardiogram prior to MRI request. | ✔ MET | An Echocardiogram Report dated January 15, 2026, is provided, confirming a prior echocardiogram was completed. |
| Criterion 3b: Echocardiogram must show LVEF (Left Ventricular Ejection Fraction) documented value. | ✔ MET | The Echocardiogram Report dated January 15, 2026, documents 'LV Ejection Fraction (LVEF) 35%'. |
| Criterion 3c: Echocardiogram must show evidence of wall motion abnormality or structural cardiac abnormality requiring further characterization. | ✔ MET | The Echocardiogram Report dated January 15, 2026, notes 'Diffuse global hypokinesis' (wall motion abnormality) and 'Moderately dilated left ventricle' (structural cardiac abnormality). |
| Criterion 4: Request must be ordered by a board-certified Cardiologist or Cardiac Electrophysiologist. | ✔ MET | EHR data specifies 'physician_specialty: Cardiology'. The Clinical Justification Note and Physician Order Letter confirm Dr. Ramesh Kumar is a 'Board Certified Cardiologist'. |
| Required Document 1: Clinical Justification Note from treating Cardiologist, explaining medical necessity, including patient history, current symptoms, previous treatments tried, reason echocardiogram is insufficient, signed and dated. | ✔ MET | The uploaded 'Clinical Justification Note' dated March 16, 2026, from Dr. Ramesh Kumar, a Board Certified Cardiologist, contains all specified elements including patient history, current symptoms, previous treatments, and a detailed explanation of echocardiogram insufficiency. It is signed and dated. |

| | | |
|---|---|---|
| Required Document 2: Most recent Echocardiogram Report within last 12 months, documenting LVEF value, chamber dimensions, wall motion analysis, and valve function. | ✔ MET | The uploaded 'Echocardiogram Report' dated January 15, 2026, is within the last 12 months (EHR filled March 18, 2026). It documents LVEF (35%), chamber dimensions (e.g., LV End Diastolic Diameter 6.2 cm), wall motion analysis (Diffuse global hypokinesis), and valve function (e.g., Mild mitral regurgitation). |
| Required Document 3: Recent Lab Results within last 30 days, including BNP or NT-proBNP levels, complete metabolic panel, complete blood count. | ✔ MET | The uploaded 'Recent Lab Results' report dated March 10, 2026, is within the last 30 days. It includes BNP (450 pg/mL), NT-proBNP (1850 pg/mL), a comprehensive metabolic panel, and a complete blood count. |
| Required Document 4: Physician Order Letter on physician letterhead, signed by referring Cardiologist, including procedure requested, ICD-10 diagnosis code, clinical indication, physician NPI number. | ✔ MET | The uploaded 'Physician Order Letter' dated March 16, 2026, from Dr. Ramesh Kumar, a Board Certified Cardiologist, is on letterhead, signed, and includes the procedure (Cardiac MRI with Contrast, CPT 75563), ICD-10 code (I42.0), clinical indication, and physician NPI (1234567890). |
| Required Document 5: Prior Treatment Records documenting cardiac medications tried for minimum 3 months, including drug names, dosages, duration of therapy, and patient response or reason for discontinuation. | ✔ MET | The uploaded 'Prior Treatment Records' dated March 16, 2026, details cardiac medications (e.g., Carvedilol, Lisinopril, Spironolactone) tried for over 3 years, including their dosages, duration of therapy, and patient response or reason for discontinuation (e.g., Sacubitril/Valsartan discontinued due to hypotension). |

## Attached Documents

The following documents are attached to this package:

| # | | |
|---|---|---|
| 1 | Clinical Justification Note Kumar | Clinical_Justification_Note_Kumar.pdf |
| 2 | Echocardiogram Report Jan2026 | Echocardiogram_Report_Jan2026.pdf |
| 3 | Laboratory Results Report | Laboratory_Results_Report.pdf |
| 4 | Physician Order Prior Auth Cardiac Mri | Physician_Order_Prior_Auth_Cardiac_MRI.pdf |
| 5 | Prior Treatment Records Cardiac Medication | Prior_Treatment_Records_Cardiac_Medication.pdf |

# City Heart Institute

123 Medical Center Drive
Phone: 555-123-4567
Department: Cardiology


Physician: Dr. Ramesh Kumar MD
NPI Number: 1234567890
Date: March 16, 2026

RE: PRIOR AUTHORIZATION REQUEST FOR CARDIAC MRI WITH CONTRAST (CPT 75563)

## PATIENT HISTORY

The patient is a 58-year-old male with a documented 3-year history of Dilated Cardiomyopathy (ICD-10: I42.0). Initially diagnosed in January 2023 following a presentation with acute dyspnea, the patient has since exhibited a chronic and progressive clinical course. Current presentation is significant for worsening shortness of breath on exertion, persistent fatigue, and a marked reduction in exercise tolerance that interferes with activities of daily living. Furthermore, the patient reports occasional palpitations, necessitating advanced diagnostic evaluation to assess for structural triggers or arrhythmogenic substrates.

## CURRENT SYMPTOMS

At present, the patient exhibits symptoms consistent with New York Heart Association (NYHA) Class III heart failure. Clinical findings include shortness of breath both at rest and upon minimal physical exertion, as well as significant orthopnea requiring 3-pillow support for sleep. Physical examination reveals bilateral lower extremity edema, indicating volume overload. Recent diagnostic monitoring confirms a persistently reduced ejection fraction, signifying a lack of improvement despite standard therapeutic interventions.

## PREVIOUS TREATMENTS TRIED

The patient has remained strictly compliant with Guideline-Directed Medical Therapy (GDMT) since diagnosis. The therapeutic regimen includes the following:

- Carvedilol: 25mg twice daily (initiated January 2023)
- Lisinopril: 10mg daily (initiated January 2023)
- Spironolactone: 25mg daily (initiated March 2023)
- Furosemide: 40mg daily for fluid management

Despite adherence to optimal medical therapy for over three years, the patient's symptoms persist and have shown recent signs of progression, suggesting the underlying pathology requires more granular characterization than what is provided by current management tools.

## CLINICAL INSUFFICIENCY OF ECHOCARDIOGRAPHY

A transthoracic echocardiogram (TTE) performed in January 2026 documented a Left Ventricular Ejection Fraction (LVEF) of 35% with global hypokinesis. While the TTE provided a baseline assessment of chamber size and function, it is insufficient to fully characterize the myocardial tissue or identify specific fibrosis patterns. Standard echocardiography lacks the resolution required to assess myocardial viability or definitively rule out infiltrative cardiomyopathies. Consequently, a Cardiac MRI with contrast is essential for definitive tissue characterization and to provide the high-resolution data necessary to guide future high-stakes treatment decisions.

## MEDICAL NECESSITY STATEMENT

Cardiac MRI with contrast is medically necessary for this patient to evaluate myocardial fibrosis via Late Gadolinium Enhancement (LGE). This study is critical to assess myocardial viability for potential revascularization candidacy and to differentiate between ischemic and non-ischemic cardiomyopathy. The findings from this advanced imaging will directly impact the clinical treatment plan, influence prognosis, and determine the appropriateness of further interventional therapies.

Respectfully,

Dr. Ramesh Kumar MD
Date Signed: March 16, 2026
Credentials: Board Certified Cardiologist
License Number: MD-12345-TX

# TRANSTHORACIC ECHOCARDIOGRAM REPORT

| | | | |
|---|---|---|---|
| **Ordering Physician:** | Dr. Ramesh Kumar MD | **Study Date:** | January 15, 2026 |
| **Performing Physician:** | Dr. Sarah Johnson MD | **Report Date:** | January 15, 2026 |
| **Facility:** | City Heart Institute | **Study Type:** | Complete TTE |

**Clinical Indication:** Dilated Cardiomyopathy; Follow up evaluation

## SECTION 1: TECHNICAL QUALITY

**Study Quality:** Adequate
**Windows:** Acceptable acoustic windows obtained.
**Views:** Parasternal long axis, short axis, apical 4-chamber, 2-chamber, 3-chamber, and subcostal views were successfully obtained and analyzed.

## SECTION 2: LEFT VENTRICLE MEASUREMENTS

| Measurement | Result | Normal Reference Range |
|---|---|---|
| LV End Diastolic Diameter | 6.2 cm | Less than 5.5 |
| LV End Systolic Diameter | 5.1 cm | Less than 3.9 |
| Interventricular Septum | 0.8 cm | 0.6 to 1.1 |
| Posterior Wall Thickness | 0.8 cm | 0.6 to 1.1 |
| **LV Ejection Fraction (LVEF)** | **35%** | Greater than 55% |
| LV Fractional Shortening | 18% | 25 to 45% |

## SECTION 3: LEFT VENTRICLE FUNCTION

**LVEF:** 35% — Severely reduced.
**Wall Motion:** Diffuse global hypokinesis noted. No regional wall motion abnormalities (RWMA) identified.
**Diastolic Function:** Grade II diastolic dysfunction (pseudonormal pattern).
**LV Size:** Moderately dilated left ventricle.

## SECTION 4: RIGHT VENTRICLE

**RV Size:** Normal.
**RV Function:** Mildly reduced.
**TAPSE:** 1.6 cm (mildly reduced).
**RV Systolic Pressure:** Estimated 38 mmHg.

## SECTION 5: VALVES

**Mitral Valve:** Mild mitral regurgitation identified. Etiology appears functional, secondary to LV dilation and annular stretching.
**Aortic Valve:** Trileaflet, normal opening. No significant stenosis or regurgitation visualized.
**Tricuspid Valve:** Mild tricuspid regurgitation.
**Pulmonic Valve:** Normal appearance and function.

## SECTION 6: OTHER FINDINGS

**Pericardium:** Small pericardial effusion noted.
**Aorta:** Normal aortic root diameter.
**IVC:** Dilated IVC suggesting elevated right atrial pressure.

## SECTION 7: CONCLUSION

1. **Severely reduced left ventricular systolic function with LVEF of 35%.**
2. **Moderately dilated left ventricle consistent with dilated cardiomyopathy.**
3. **Diffuse global hypokinesis.**
4. **Mild mitral regurgitation — functional etiology.**
5. **Grade II diastolic dysfunction.**
6. **Small pericardial effusion.**
7. **Recommend Cardiac MRI for further tissue characterization.**

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**Interpreting Physician:** Dr. Sarah Johnson MD
Board Certified Echocardiographer
Date: January 15, 2026

# CITY HEART INSTITUTE

**Clinical Pathology Laboratory**

## Laboratory Results Report

Clinical Laboratory Report

| | | | |
|---|---|---|---|
| **Ordering Physician:** | Dr. Ramesh Kumar MD | **Specimen Type:** | Venous Blood |
| **Collection Date:** | March 10, 2026 | **Clinical Indication:** | Dilated Cardiomyopathy |
| **Report Date:** | March 10, 2026 | | Cardiac MRI Pre-authorization |

## SECTION 1 - CARDIAC BIOMARKERS

| Test | Result | Unit | Reference Range | Flag |
|---|---|---|---|---|
| BNP (B-type Natriuretic Peptide) | **450** | pg/mL | Less than 100 | **HIGH** |
| NT-proBNP | **1850** | pg/mL | Less than 125 (age < 75) | **HIGH** |
| Troponin I (High Sensitivity) | 0.04 | ng/mL | Less than 0.04 | **BORDERLINE** |

## SECTION 2 - COMPLETE BLOOD COUNT

| Test | Result | Unit | Reference Range | Flag |
|---|---|---|---|---|
| WBC | 7.2 | 10^3/uL | 4.5 to 11.0 | NORMAL |
| RBC | 4.8 | 10^6/uL | 4.5 to 5.9 | NORMAL |
| Hemoglobin | 13.8 | g/dL | 13.5 to 17.5 | NORMAL |
| Hematocrit | 41.2 | % | 41 to 53 | NORMAL |
| Platelets | 215 | 10^3/uL | 150 to 400 | NORMAL |
| MCV | 88 | fL | 80 to 100 | NORMAL |

## SECTION 3 - COMPREHENSIVE METABOLIC PANEL

| Test | Result | Unit | Reference Range | Flag |
|---|---|---|---|---|
| Sodium | 138 | mEq/L | 136 to 145 | NORMAL |
| Potassium | 4.1 | mEq/L | 3.5 to 5.1 | NORMAL |
| Creatinine | 1.1 | mg/dL | 0.7 to 1.3 | NORMAL |
| eGFR | 72 | mL/min | Greater than 60 | NORMAL |
| BUN | 18 | mg/dL | 7 to 20 | NORMAL |

| Test | Result | Unit | Reference Range | Flag |
|---|---|---|---|---|
| Glucose | 95 | mg/dL | 70 to 100 | NORMAL |
| AST | 28 | U/L | 10 to 40 | NORMAL |
| ALT | 22 | U/L | 7 to 56 | NORMAL |
| Albumin | 3.8 | g/dL | 3.5 to 5.0 | NORMAL |

## SECTION 4 - LIPID PANEL

| Test | Result | Unit | Reference Range | Flag |
|---|---|---|---|---|
| Total Cholesterol | 195 | mg/dL | Less than 200 | NORMAL |
| LDL | 118 | mg/dL | Less than 130 | NORMAL |
| HDL | 42 | mg/dL | Greater than 40 | NORMAL |
| Triglycerides | **175** | mg/dL | Less than 150 | **BORDERLINE HIGH** |

## SECTION 5 - INTERPRETATION NOTE

Significantly elevated BNP at 450 pg/mL and NT-proBNP at 1850 pg/mL consistent with moderate to severe heart failure.
Renal function preserved. Electrolytes within normal limits.
Results support ongoing heart failure management and prior authorization for advanced cardiac imaging.

**Authorized By:**

**Dr. Michael Chen MD**
Board Certified Pathologist
Date: March 10, 2026

# CITY HEART INSTITUTE

Department of Cardiology | 123 Medical Center Drive | Houston, Texas 77001
Phone: 555-123-4567 | Fax: 555-123-4568 | www.cityheartinstitute.com

Date: March 16, 2026

To:
United Healthcare Prior Authorization Department
Radiology Prior Authorization Team

From:
Dr. Ramesh Kumar MD
Board Certified Cardiologist
NPI: 1234567890 | License: MD-12345-TX | DEA: RK1234563

SUBJECT: PRIOR AUTHORIZATION REQUEST
PROCEDURE: CARDIAC MRI WITH CONTRAST (CPT 75563)
ICD-10 CODE: I42.0

## REQUESTED PROCEDURE

| | |
|---|---|
| Name: | Cardiac MRI with Contrast |
| CPT Code: | 75563 |
| Urgency: | Routine — Non Emergency |
| Facility: | City Heart Institute Radiology |
| Estimated Date: | Within 30 days of approval |

## CLINICAL INDICATION

Primary Diagnosis: Dilated Cardiomyopathy (ICD-10: I42.0)
Secondary Diagnosis: Heart Failure with Reduced Ejection Fraction (ICD-10: I50.20)

## MEDICAL NECESSITY STATEMENT

This procedure is medically necessary for:

- Tissue characterization of myocardium
- Assessment of myocardial fibrosis via late gadolinium enhancement imaging
- Differentiation of ischemic versus non-ischemic cardiomyopathy
- Assessment of myocardial viability
- Guidance of treatment decisions including consideration of ICD implantation

## SUPPORTING CLINICAL INFORMATION

- LVEF 35% on recent echocardiogram
- Elevated BNP 450 pg/mL
- NYHA Class III symptoms
- Optimal medical therapy ongoing
- No contraindications to MRI or gadolinium contrast media

> I certify that this procedure is medically necessary and appropriate for this patient. All information provided is accurate and complete to the best of my knowledge.

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Dr. Ramesh Kumar MD
Board Certified Cardiologist
Date: March 16, 2026
NPI: 1234567890
Contact: 555-123-4567

# CITY HEART INSTITUTE

**Cardiology Department**

## Prior Treatment Records

**Cardiac Medication History**

| | | | |
|---|---|---|---|
| **Treating Physician:** | Dr. Ramesh Kumar MD | **Report Date:** | March 16, 2026 |
| **Facility:** | City Heart Institute | **Specialty:** | Cardiology |
| **Period Covered:** | Jan 2023 – Mar 2026 | **Clinical Indication:** | Dilated Cardiomyopathy |

*This document summarizes the complete cardiac medication history for a patient with confirmed Dilated Cardiomyopathy (ICD-10: I42.0) over a period of 3 years from January 2023 to March 2026. All medications listed below were prescribed and monitored by the treating cardiologist as part of optimal medical therapy for heart failure with reduced ejection fraction.*

## SECTION 1 - MEDICATION HISTORY TABLE

| Medication | Dose/ Freq | Dates | Duration | Response | Status |
|---|---|---|---|---|---|
| **Carvedilol** (Beta Blocker) | 25mg Twice daily | Jan 2023 – Present | 3 years | Partial response — HR controlled but symptoms persist | CURRENT |
| **Lisinopril** (ACE Inhibitor) | 10mg Once daily | Jan 2023 – Present | 3 years | Partial response — BP controlled but LVEF unchanged | CURRENT |
| **Spironolactone** (Aldo. Antagonist) | 25mg Once daily | Mar 2023 – Present | 2y 9mo | Minimal improvement in symptoms | CURRENT |
| **Furosemide** (Loop Diuretic) | 40mg Once daily | Mar 2023 – Present | 2y 9mo | Adequate fluid control; edema managed | CURRENT |
| **Sacubitril/ Valsartan** (ARNI) | 24/26mg Twice daily | Sep 2023 – Nov 2023 | 2 months | Discontinued due to symptomatic hypotension and dizziness | DISCONTINUED |
| **Ivabradine** | 5mg Twice daily | Jan 2024 – Mar 2024 | 2 months | Discontinued — inadequate heart rate reduction | DISCONTINUED |

## SECTION 2 - TREATMENT RESPONSE SUMMARY

Despite 3 years of optimal medical therapy with maximum tolerated doses of guideline directed medical therapy (GDMT) including beta blocker, ACE inhibitor, and aldosterone antagonist, patient continues to exhibit:

- Persistent LVEF of 35%
- NYHA Class III symptoms (marked limitation in activity)
- Elevated BNP levels (refer to lab report dated 03/10/2026)
- Reduced functional capacity

## SECTION 3 - CURRENT MEDICATION LIST

- **Carvedilol:** 25mg twice daily
- **Lisinopril:** 10mg once daily
- **Spironolactone:** 25mg once daily
- **Furosemide:** 40mg once daily
- **Aspirin:** 81mg once daily
- **Atorvastatin:** 40mg once daily

**Physician Attestation:**
I certify that the above medication history is accurate and complete. The patient has received optimal guideline directed medical therapy for a minimum of 3 years with inadequate clinical response. Advanced cardiac imaging (Cardiac MRI) is required to guide further treatment decisions, including viability assessment and potential ICD placement.

**Dr. Ramesh Kumar MD**
Board Certified Cardiologist | NPI: 1234567890
Date: March 16, 2026